EL SEDENTARISMO

Una investigación sobre su impacto

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PREGUNTA PECO

En **niños y adolescentes** de 5 a 17 años (P), ¿pasar más de **2 horas al día viendo televisión u otras pantallas** (E), en **comparación con pasar menos de 2 horas al día** (C), se asocia con un **mayor riesgo** de **sobrepeso u obesidad** (O)?



DEFINICIÓN DE VARIABLES

Sedentarismo

El sedentarismo o comportamiento sedentario se asocia con un **estilo de vida** caracterizado por una **baja actividad física** diaria y un **gasto de energía mínimo** durante las horas de vigilia, habitualmente ligado a actividades que se realizan sentado o recostado (2)



Obesidad

La obesidad se describe clínicamente como una **enfermedad crónica y multifactorial** que consiste en una **acumulación excesiva de grasa en el cuerpo**, lo cual altera la composición corporal normal y afecta de forma directa el estado de salud (1)



ARTÍCULO 1: DETERMINANTES NUTRICIONALES Y CONDUCTUALES DE LA OBESIDAD EN ADOLESCENTES: UN ESTUDIO DE CASOS Y CONTROLES EN SRI LANKA

Revista: BMC Public Health

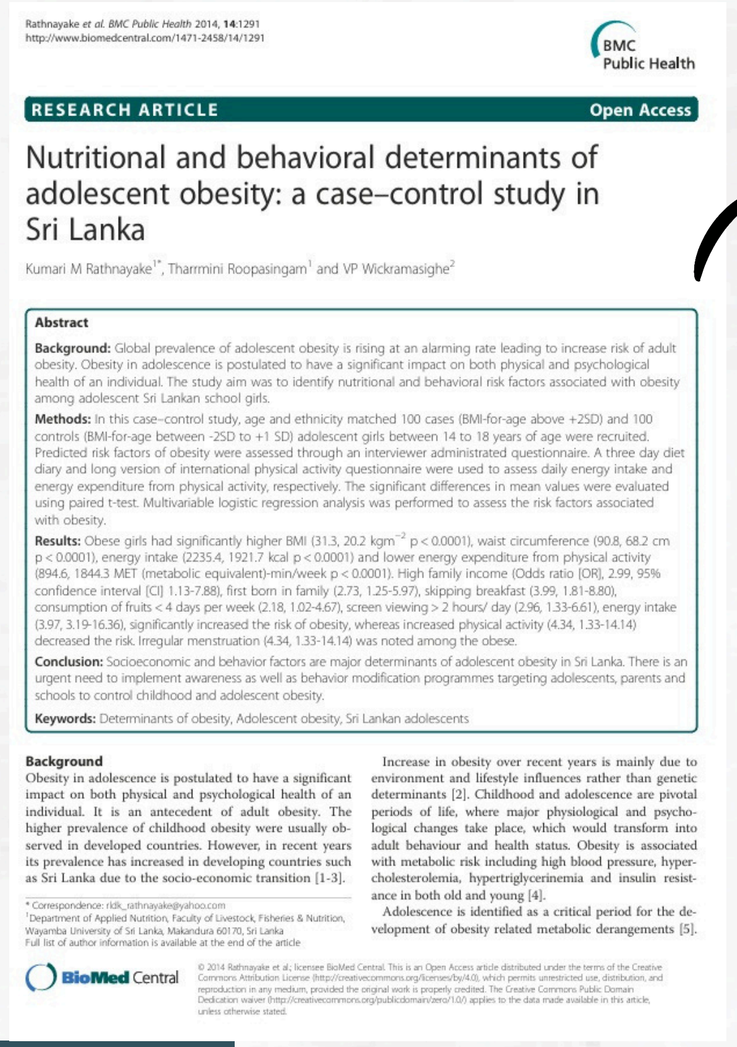
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INTRODUCCIÓN

El estudio evalúa directamente el impacto del tiempo de uso de pantallas (screen viewing time) sobre el riesgo de desarrollar obesidad en adolescentes.

Tipo de artículo: artículo de investigación original de tipo caso-control

Objetivo: Identificar factores nutricionales y conductuales asociados con la obesidad adolescente.



Rathnayake *et al. BMC Public Health* 2014, **14**:1291
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BMC Public Health

RESEARCH ARTICLE Open Access

Nutritional and behavioral determinants of adolescent obesity: a case–control study in Sri Lanka

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Abstract

Background: Global prevalence of adolescent obesity is rising at an alarming rate leading to increase risk of adult obesity. Obesity in adolescence is postulated to have a significant impact on both physical and psychological health of an individual. The study aim was to identify nutritional and behavioral risk factors associated with obesity among adolescent Sri Lankan school girls.

Methods: In this case–control study, age and ethnicity matched 100 cases (BMI-for-age above +2SD) and 100 controls (BMI-for-age between -2SD to +1 SD) adolescent girls between 14 to 18 years of age were recruited. Predicted risk factors of obesity were assessed through an interviewer administrated questionnaire. A three day diet diary and long version of international physical activity questionnaire were used to assess daily energy intake and energy expenditure from physical activity, respectively. The significant differences in mean values were evaluated using paired t-test. Multivariable logistic regression analysis was performed to assess the risk factors associated with obesity.

Results: Obese girls had significantly higher BMI (31.3, 20.2 kgm^{-2} $p < 0.0001$), waist circumference (90.8, 68.2 cm $p < 0.0001$), energy intake (2235.4, 1921.7 kcal $p < 0.0001$) and lower energy expenditure from physical activity (894.6, 1844.3 MET (metabolic equivalent)-min/week $p < 0.0001$). High family income (Odds ratio [OR], 2.99, 95% confidence interval [CI] 1.13-7.88), first born in family (2.73, 1.25-5.97), skipping breakfast (3.99, 1.81-8.80), consumption of fruits < 4 days per week (2.18, 1.02-4.67), screen viewing > 2 hours/ day (2.96, 1.33-6.61), energy intake (3.97, 3.19-16.36), significantly increased the risk of obesity, whereas increased physical activity (4.34, 1.33-14.14) decreased the risk. Irregular menstruation (4.34, 1.33-14.14) was noted among the obese.

Conclusion: Socioeconomic and behavior factors are major determinants of adolescent obesity in Sri Lanka. There is an urgent need to implement awareness as well as behavior modification programmes targeting adolescents, parents and schools to control childhood and adolescent obesity.

Keywords: Determinants of obesity, Adolescent obesity, Sri Lankan adolescents

Background

Obesity in adolescence is postulated to have a significant impact on both physical and psychological health of an individual. It is an antecedent of adult obesity. The higher prevalence of childhood obesity were usually observed in developed countries. However, in recent years its prevalence has increased in developing countries such as Sri Lanka due to the socio-economic transition [1-3].

Increase in obesity over recent years is mainly due to environment and lifestyle influences rather than genetic determinants [2]. Childhood and adolescence are pivotal periods of life, where major physiological and psychological changes take place, which would transform into adult behaviour and health status. Obesity is associated with metabolic risk including high blood pressure, hypercholesterolemia, hypertriglycerinemia and insulin resistance in both old and young [4].

Adolescence is identified as a critical period for the development of obesity related metabolic derangements [5].

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METODOLOGÍA

- **Población y muestra:** 200 adolescentes mujeres de 14 a 18 años (100 con obesidad y 100 controles), de Sri Lanka.
    - **Grupo Casos:** Adolecentes con obesidad (IMC para la edad > +2 SD)
    - **Grupo Casos:** Adolescentes con peso normal (IMC para la edad entre -2 SD y +1 SD)
- **Variables evaluadas:**

1. **V.I (Exposición):** Tiempo de visualización de pantallas mayor a 2 horas al día (evaluado mediante cuestionarios) además de IPAQ.
2. **V. D (Desenlace):** Presencia de obesidad (medida mediante IMC y circunferencia de cintura).

- **Análisis estadístico:** Se emplearon pruebas de t de Student para variables continuas y análisis de regresión logística univariable y multivariable para los (OR) con sus intervalos de confianza

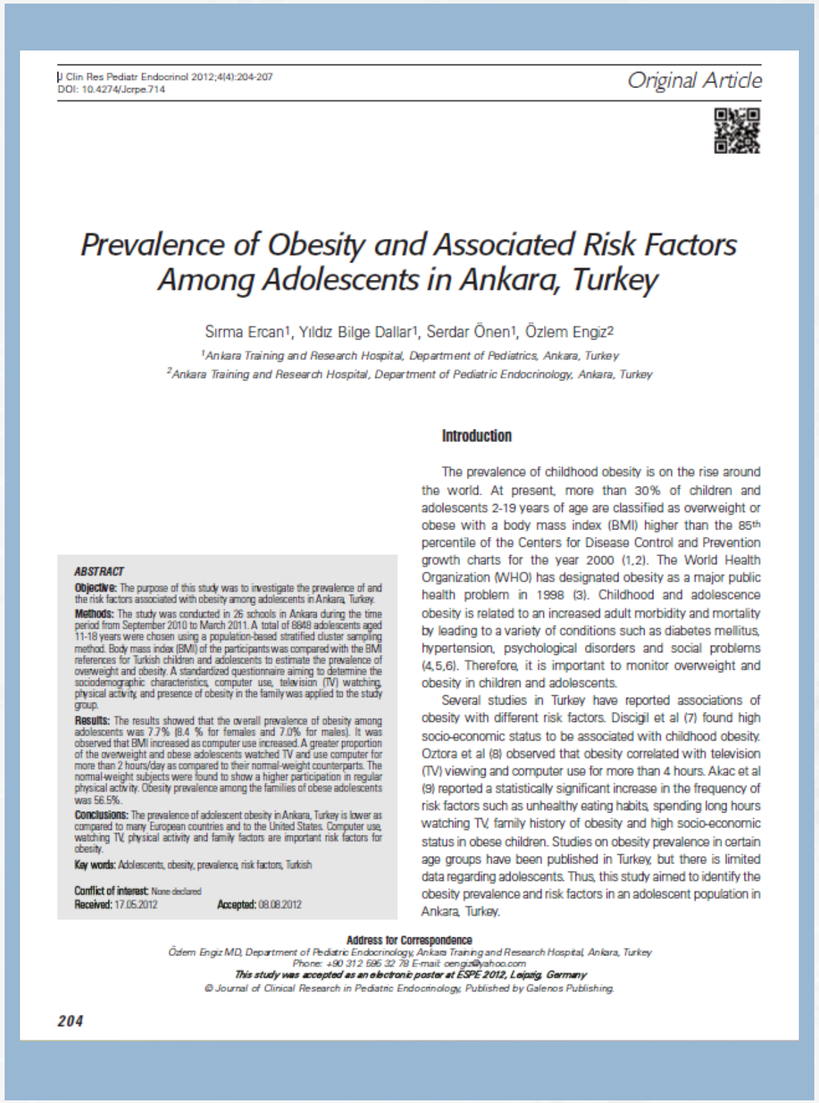
ARTÍCULO 1: DETERMINANTES NUTRICIONALES Y CONDUCTUALES DE LA OBESIDAD EN ADOLESCENTES: UN ESTUDIO DE CASOS Y CONTROLES EN SRI LANKA

Table 2 Potential risk factors associated with adolescent obesity

Factor	Univariable analysis		Multivariable analysis		
	OR	P value	OR	P value	95% Confidence interval[a]
Screen viewing hours/day (>2 hours/day)	3.34	0.0002	2.96	0.008	1.33-6.61

[26]. Our study, showed that >2 hours of screen time per day had three times higher risk of becoming obese.

ARTÍCULO 2: PREVALENCE OF OBESITY AND ASSOCIATED RISK FACTORS AMONG ADOLESCENTS IN ANKARA, TURKEY



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Original Article

Prevalence of Obesity and Associated Risk Factors Among Adolescents in Ankara, Turkey

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ABSTRACT

Objective: The purpose of this study was to investigate the prevalence of and the risk factors associated with obesity among adolescents in Ankara, Turkey.

Methods: The study was conducted in 26 schools in Ankara during the time period from September 2010 to March 2011. A total of 8848 adolescents aged 11-18 years were chosen using a population-based stratified cluster sampling method. Body mass index (BMI) of the participants was compared with the BMI references for Turkish children and adolescents to estimate the prevalence of overweight and obesity. A standardized questionnaire aiming to determine the sociodemographic characteristics, computer use, television (TV) watching, physical activity, and presence of obesity in the family was applied to the study group.

Results: The results showed that the overall prevalence of obesity among adolescents was 7.7% (8.4 % for females and 7.0% for males). It was observed that BMI increased as computer use increased. A greater proportion of the overweight and obese adolescents watched TV and use computer for more than 2 hours/day as compared to their normal-weight counterparts. The normal-weight subjects were found to show a higher participation in regular physical activity. Obesity prevalence among the families of obese adolescents was 56.5%.

Conclusions: The prevalence of adolescent obesity in Ankara, Turkey is lower as compared to many European countries and to the United States. Computer use, watching TV, physical activity and family factors are important risk factors for obesity.

Key words: Adolescents, obesity, prevalence, risk factors, Turkish

Conflict of interest: None declared
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Introduction

The prevalence of childhood obesity is on the rise around the world. At present, more than 30% of children and adolescents 2-19 years of age are classified as overweight or obese with a body mass index (BMI) higher than the 85th percentile of the Centers for Disease Control and Prevention growth charts for the year 2000 (1,2). The World Health Organization (WHO) has designated obesity as a major public health problem in 1998 (3). Childhood and adolescence obesity is related to an increased adult morbidity and mortality by leading to a variety of conditions such as diabetes mellitus, hypertension, psychological disorders and social problems (4,5,6). Therefore, it is important to monitor overweight and obesity in children and adolescents.

Several studies in Turkey have reported associations of obesity with different risk factors. Discigil et al (7) found high socio-economic status to be associated with childhood obesity. Oztora et al (8) observed that obesity correlated with television (TV) viewing and computer use for more than 4 hours. Akac et al (9) reported a statistically significant increase in the frequency of risk factors such as unhealthy eating habits, spending long hours watching TV, family history of obesity and high socio-economic status in obese children. Studies on obesity prevalence in certain age groups have been published in Turkey, but there is limited data regarding adolescents. Thus, this study aimed to identify the obesity prevalence and risk factors in an adolescent population in Ankara, Turkey.

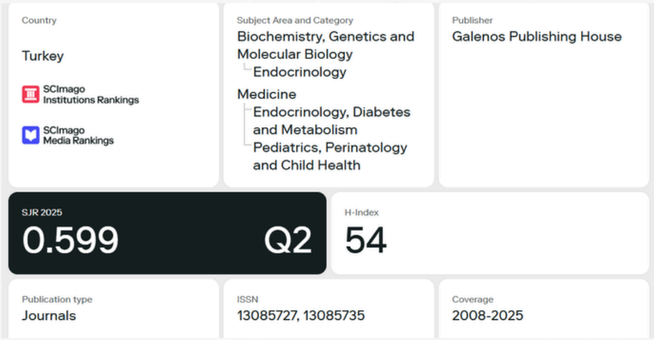
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1. OBJETIVO

El propósito de este estudio fue investigar la prevalencia y los factores de riesgo asociados con la obesidad entre los adolescentes en Ankara, Turquía

2. DISEÑO

MUESTRA POBLACIONAL: Se seleccionaron un total de 8848 adolescentes de 11 a 18 años
DISEÑO: Técnica de muestreo por conglomerados aleatorios multietapa, donde dividieron distritos escolares y seleccionaron aleatoriamente 26 escuelas
VARIABLE (EXPOSICIÓN): Tiempo diario frente a la televisión y uso de computadoras
VARIABLE (DESENLACE): Prevalencia de sobrepeso u obesidad determinada mediante el cálculo del Índice de Masa Corporal (IMC)

- **TIPO DE ESTUDIO:** estudio de prevalencia o de corte transversal (observacional y descriptiva).
- **REFERENCIA:** Se establecen los puntos de corte numéricos exactos del IMC para cada edad y sexo biológico.

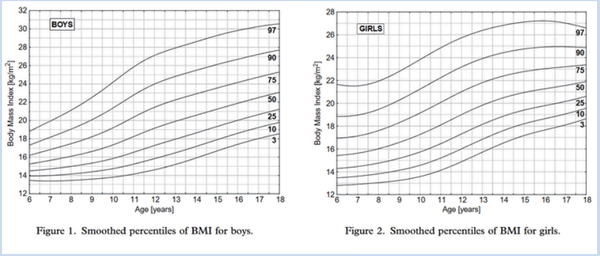


Figure 1. Smoothed percentiles of BMI for boys.

Figure 2. Smoothed percentiles of BMI for girls.

ARTÍCULO 2: PREVALENCE OF OBESITY AND ASSOCIATED RISK FACTORS AMONG ADOLESCENTS IN ANKARA, TURKEY

3. RESULTADOS

- **Prueba de Chi-cuadrado:** Sirvió para comparar porcentajes, como la cantidad exacta de adolescentes con peso normal frente a aquellos con sobrepeso u obesidad que superaban las 2 horas de pantalla.
- **Análisis de Varianza (ANOVA):** Esta prueba estadística permitió comparar numéricamente los promedios (las medias) entre tres o más grupos independientes dentro de la gran muestra poblacional.

Table 1. The distribution of obesity with respect to age and sex

Variables	Normal weight n (%)	Overweight n (%)	Obese n (%)	p
Age				<0.001
11-14 years	4036 (87)	329 (7.1)	273 (5.9)	
15-18 years	3397 (80.7)	408 (9.7)	405 (9.6)	
Sex				<0.001
Girls	3587 (81.1)	466 (10.6)	369 (8.4)	
Boys	3860 (86.9)	271 (6.1)	309 (7)	

- **Prevalencia de obesidad:** se identificó obesidad en el 7,7 % de los adolescentes estudiados.
- **Diferencias por sexo:** la prevalencia fue mayor en las chicas (8,4 %) que en los chicos (7,0 %).
- **Actividad física:** el 53,7 % de los participantes no realizaba actividad física según el criterio utilizado por los investigadores.
- **Antecedentes familiares:** entre los adolescentes con obesidad, el 56,5 % tenía antecedentes familiares de obesidad.
- **Tiempo frente a pantallas:** los adolescentes con sobrepeso u obesidad tendían a utilizar la computadora y ver televisión durante más de dos horas al día.
- **Edad:** la obesidad fue más frecuente en el grupo de 15 a 18 años (9,6 %) que en el de 11 a 14 años (5,9 %).

CONLUSIONES

Table 2. Relationships between obesity and computer use, television (TV) watching, physical activity and family history of obesity

Variables	Normal weight (n=7433)	Overweight (n=737)	Obese (n=678)	p
Computer use	5781 (77.8)	631 (85.6)	652 (96.2)	<0.001
Computer use time				
1 hour/day	3244 (56.1)	238 (37.7)	167 (25.6)	
2 hours/day	1510 (26.1)	128 (20.3)	300 (46.0)	
>2 hours/day	1027 (17.8)	265 (42.0)	185 (28.4)	<0.001
TV watching				
Do not watch	368 (5.0)	15 (2.0)	13 (1.9)	
1 hour/day	3363 (45.2)	260 (35.3)	191 (28.2)	
2 hours/day	2431 (32.7)	249 (33.8)	320 (47.2)	
>2 hours/day	1271 (17.1)	213 (28.9)	154 (22.7)	<0.001
Physical activity				
No activity	3944 (53.1)	408 (55.4)	396 (58.4)	
Irregular	1161 (15.6)	119 (16.1)	100 (14.8)	
Regular	2328 (31.3)	210 (28.5)	182 (26.8)	0.048
Family history of obesity	1415 (19.0)	216 (29.3)	383 (56.5)	<0.001

LIMITACIONES

- **Diseño transversal:** (Cross-sectional study): Mde la exposición y el desenlace en un mismo momento, no permite establecer una relación de causalidad definitiva
- **Uso de cuestionarios autorreportados**: El tiempo de visualización de televisión y el uso de computadoras fueron informados por los propios adolescentes mediante encuestas (sesgo).

CONCLUSIONES:

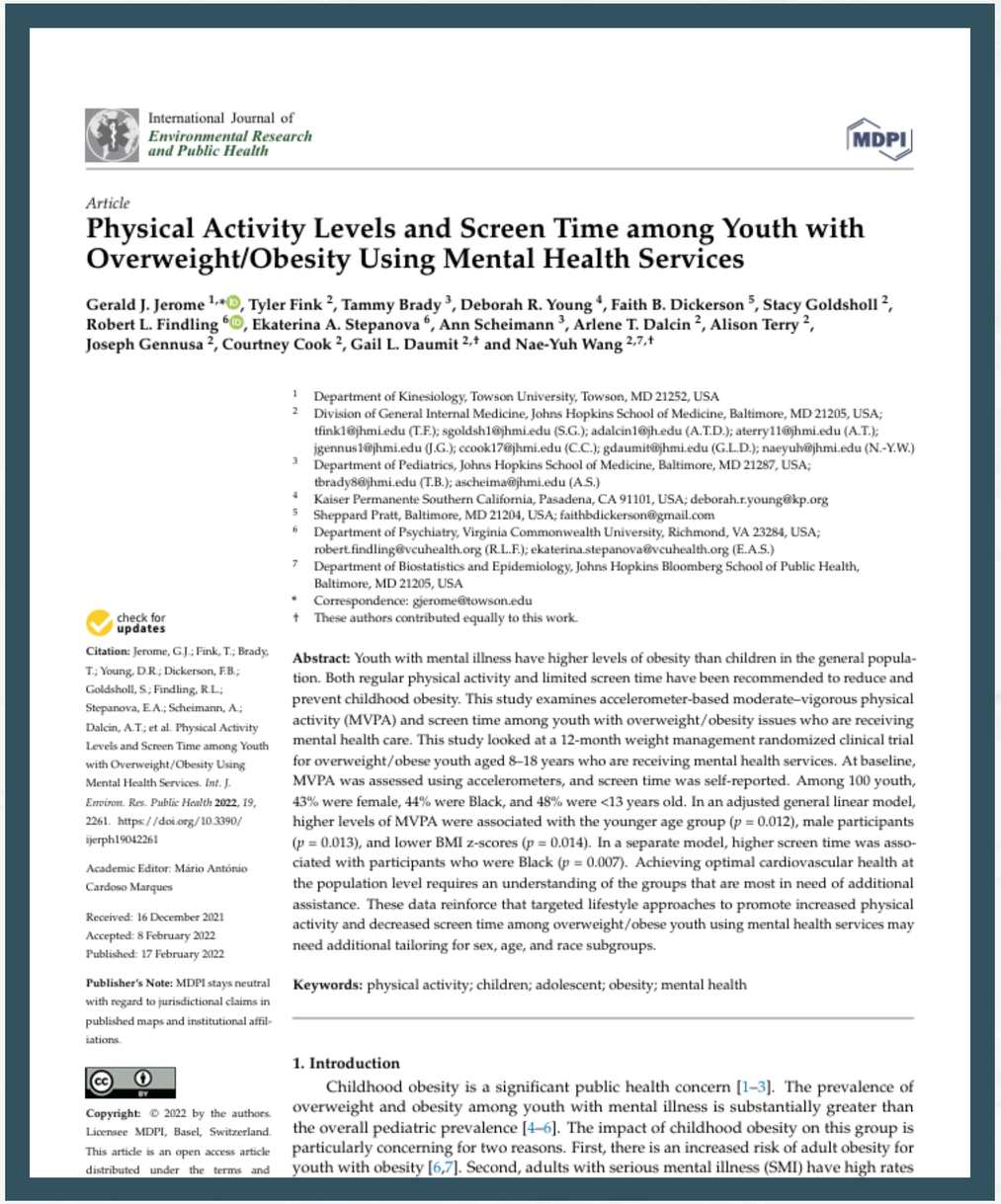
- La prevalencia de obesidad adolescente en Ankara, Turquía, es menor que la registrada en muchos países europeos y en Estados Unidos.
- El uso de la computadora, ver televisión, la actividad física y los factores familiares son factores de riesgo importantes para la obesidad.

El uso de la computadora fue más relevante en niños obesos y con sobrepeso

Sí, la evidencia científica demuestra consistentemente que pasar más de 2 horas al día frente a pantallas se asocia con un mayor riesgo de desarrollar sobrepeso u obesidad en la población infanto-juvenil.

ARTÍCULO 3:

Physical Activity Levels and screen time among youth with overweight/obesity using mental health services



International Journal of *Environmental Research and Public Health*

MDPI

Article

Physical Activity Levels and Screen Time among Youth with Overweight/Obesity Using Mental Health Services

Gerald J. Jerome [1,*], Tyler Fink [2], Tammy Brady [3], Deborah R. Young [4], Faith B. Dickerson [5], Stacy Goldsholl [2], Robert L. Findling [6], Ekaterina A. Stepanova [6], Ann Scheimann [3], Arlene T. Dalcin [2], Alison Terry [2], Joseph Gennusa [2], Courtney Cook [2], Gail L. Daumit [2,†] and Nae-Yuh Wang [2,7,†]

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Abstract: Youth with mental illness have higher levels of obesity than children in the general population. Both regular physical activity and limited screen time have been recommended to reduce and prevent childhood obesity. This study examines accelerometer-based moderate–vigorous physical activity (MVPA) and screen time among youth with overweight/obesity issues who are receiving mental health care. This study looked at a 12-month weight management randomized clinical trial for overweight/obese youth aged 8–18 years who are receiving mental health services. At baseline, MVPA was assessed using accelerometers, and screen time was self-reported. Among 100 youth, 43% were female, 44% were Black, and 48% were <13 years old. In an adjusted general linear model, higher levels of MVPA were associated with the younger age group ($p = 0.012$), male participants ($p = 0.013$), and lower BMI z-scores ($p = 0.014$). In a separate model, higher screen time was associated with participants who were Black ($p = 0.007$). Achieving optimal cardiovascular health at the population level requires an understanding of the groups that are most in need of additional assistance. These data reinforce that targeted lifestyle approaches to promote increased physical activity and decreased screen time among overweight/obese youth using mental health services may need additional tailoring for sex, age, and race subgroups.

Keywords: physical activity; children; adolescent; obesity; mental health

1. Introduction

Childhood obesity is a significant public health concern [1–3]. The prevalence of overweight and obesity among youth with mental illness is substantially greater than the overall pediatric prevalence [4–6]. The impact of childhood obesity on this group is particularly concerning for two reasons. First, there is an increased risk of adult obesity for youth with obesity [6,7]. Second, adults with serious mental illness (SMI) have high rates

International Journal of Environmental Research and Public Health

Su cuartil es Q1 en 2021 y Q2 en 2022 según SJR y el factor de impacto es 4,614 de acuerdo con JCR

Artículo de investigación original

Análisis transversal de datos basales de un ensayo clínico aleatorizado de manejo de peso (NCT03075306)



100 jóvenes de 8 a 18 años con sobrepeso u obesidad (de 112 inscritos) que reciben atención de salud mental.

Objetivo Principal

Describe la actividad física (medida con acelerómetro) y el tiempo de pantalla (autorreporte) en jóvenes de 8 a 18 años con sobrepeso u obesidad que reciben atención de salud mental.

Niveles de Actividad Física y Tiempo de Pantalla según Grupo de Etaria (Edad)

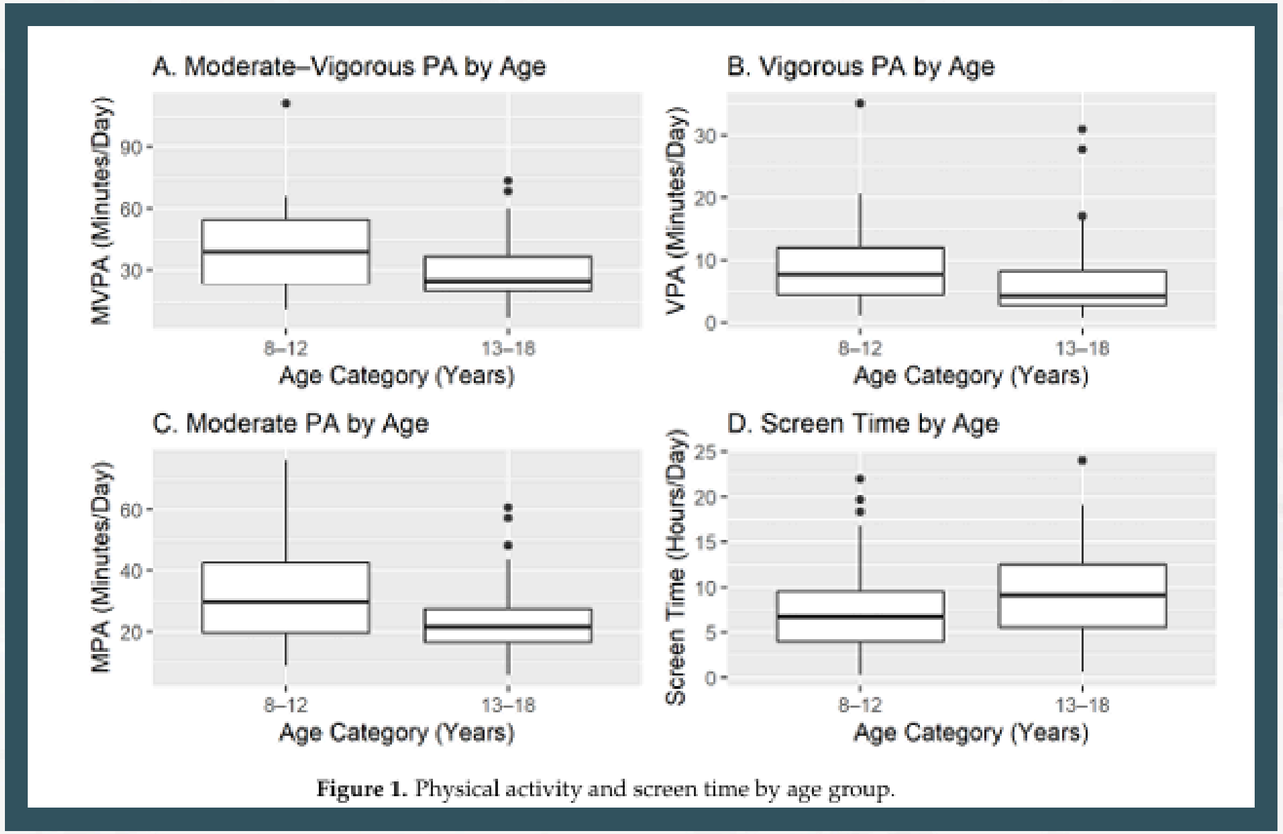


Figure 1. Physical activity and screen time by age group.

Conclusiones de las gráficas

Los jóvenes con obesidad y problemas de salud mental presentan niveles de actividad física críticamente bajos y un tiempo de pantalla alarmante (promedio de 8.5 horas diarias).

La falta de movimiento se acentúa en adolescentes, mujeres, jóvenes con mayor IMC y pacientes con ansiedad, mientras que el sedentarismo digital afecta con mayor fuerza a la población afrodescendiente

Subgráfico A: Actividad Física Moderada a Vigorosa (MVPA - Minutes/Day)

Muestra los minutos totales de actividad física moderada a intensa al día.

Los niños de 8 a 12 años tienen un promedio significativamente mayor (39.9 min/día) en comparación con los adolescentes de 13 a 18 años (29.7 min/día), con un valor de p = 0.005.

Subgráfico B: Actividad Física Vigorosa (VPA - Minutes/Day)

Mide la actividad física de alta intensidad (correr rápido, deportes pesados).

Promedio general: 8.8 min en niños vs. 6.6 min en adolescentes. No mostró una diferencia estadísticamente drástica según la prueba t tradicional (p = 0.080).

Subgráfico C: Actividad Física Moderada (MPA - Minutes/Day)

Mide ejercicios de intensidad moderada (caminar rápido, trote ligero).

Los niños de 8 a 12 años promedian 31.2 minutos frente a los 23.1 minutos de los de 13-18 años ($p = 0.002$).

Subgráfico D: Tiempo de Pantalla (Screen Time - Hours/Day)

Mide las horas diarias frente a dispositivos (TV, celulares, computadoras, videojuegos).

Los niños de 8-12 años promedian 7.6 horas/día y los adolescentes de 13-18 años promedian 9.5 horas/día.

Factores asociados a la actividad física y el tiempo de pantalla (Modelo Multivariable GLM)

Table 2. Association of physical activity and screen time with participant characteristics.

Participant Characteristic	Moderate-to-Vigorous Physical Activity			Screen Time		
	β	95% CI	p [1]	β	95% CI	p [1]
Intercept	54.2	(37.8, 70.6)	<0.001	0.6	(−4.5, 5.6)	0.829
Age Group						
8–12 years	ref			ref		
13–18 years	−8.5	(−15.1, −2.0)	0.012	1.9	(−0.1, 3.9)	0.063
Sex						
Female	ref			ref		
Male	9.4	(2.1, 16.6)	0.013	1.9	(−0.3, 4.1)	0.101
Race						
Non-Black	ref			ref		
Black	4.4	(−2.0, 10.9)	0.179	2.8	(0.8, 4.7)	0.007
Diagnosis			0.034 [2]			0.345 [2]
ADHD	ref			ref		
Anxiety	−9.8	(−17.7, −1.9)		1.8	(−0.6, 4.2)	
Depression	−1.6	(−11.0, 7.8)		1.0	(−1.9, 3.8)	
BMI z-score	−9.5	(−16.9, −2.1)	0.014	1.9	(−0.04, 4.2)	0.102

[1] p-Values represent pairwise comparison with the reference category. [2] Overall test examining chi-square p-value with d.f. = 2.

Conclusiones de la tabla

Sedentarismo Crítico: Los jóvenes con sobrepeso/obesidad y problemas de salud mental presentan niveles de actividad física alarmantemente bajos y un tiempo de pantalla excesivo (promedio superior a 8 horas diarias).

Factores de Riesgo Diferenciados: La falta de movimiento se agrava en adolescentes, mujeres, jóvenes con mayor IMC y pacientes con ansiedad, mientras que el sedentarismo digital afecta con mayor fuerza a la población afrodescendiente

Reflexión Estratégica: Se requieren intervenciones médicas y psicológicas tempranas, personalizadas y culturalmente adaptadas que aborden de manera integral la salud física y mental.

Factores que afectan la Actividad Física Moderada a Vigorosa (MVPA)

Edad (β = -8.5, p = 0.012): Los adolescentes de 13 a 18 años realizan significativamente menos actividad física que los niños de 8 a 12 años.

Sexo (β= 9.4, p = 0.013): Los varones presentan un nivel de actividad física mayor en comparación con las mujeres dentro de este grupo clínico.

Índice de Masa Corporal / IMC (β= -9.5, p = 0.014): Un mayor puntaje de IMC (mayor grado de obesidad) se asocia directamente con una menor actividad física.

Diagnóstico Psiquiátrico (p = 0.034): Tomando el TDAH (ADHD) como referencia, los jóvenes que sufren de ansiedad realizan una menor cantidad de actividad física en comparación.

Factores que afectan el Tiempo de Pantalla (Screen Time)

Raza / Etnicidad (β = 2.8, p = 0.007): Fue la única variable que mostró un impacto estadísticamente contundente en el uso de pantallas.

Los participantes autoidentificados como afrodescendientes (Black) reportaron un tiempo de pantalla significativamente mayor (casi 3 horas más al día) en comparación con el grupo de referencia (non-Black).

Las demás variables (edad, sexo o diagnósticos) no mostraron diferencias estadísticamente drásticas en el tiempo de pantalla dentro de este modelo.

GRACIAS

